



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض رابعياً : صفيه عبدالمنعم بهي الدماطي

المحافظة : القليوبية الحي / المدينة : شبين القناطر

رقم البطاقة : 26208221300929 رقم التليفون : 01029245973

العمر (تاريخ الميلاد) : 22-08-1962 المهنة : ربه منزل

العنوان : ممممممممممممم

تاريخ تسجيل الإستمارة : 23-09-2026

### الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثالث اكسيمستان قسم العلاج : [14503]

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ..... التوقيع : .....  
الاسم : د/ندى حسن الوظيفة : ..... التوقيع : .....  
الاسم : د/نرمين رمضان الوظيفة : ..... التوقيع : .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. .... /د. .... /د. .... /د. ....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : ..... قسم : .....  
جهة العلاج : ..... نفقات : .....  
ملاحظات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

Name: صفية عبدالمنعم يحيى الدماطي  
National ID: 26208221300929



## تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :  
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :  
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Exemestane hormonal therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :  
د/شيماء لاشين

جمهورية مصر العربية  
بطاقة تعريف شخصية

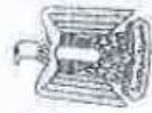
صفحه  
عبد النعم بنهي المنطلي  
عمر اشرف القبلي  
مركز شبين القناطر - القنوية

٢٦٢٠٨٢٢١٣٠٠٩٢٩



46451

١٠٤١/١٠٠ ٤٤٢٠٨٢٢١٣٠٠٩٢٩  
زينة مراد  
أنثى  
مسلمة  
ارملة



الطاقة سارية حتى ٢٠٢٩/١٠/٢٢

١٦





مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية



06/28/2026

**MDT FORM**

|                |                                  |                 |                |
|----------------|----------------------------------|-----------------|----------------|
| Name:          | Safya Abde Elminam Bahi Eldamate | MRN:            | 7009           |
| Age:           | 64 Y                             | ID:             | 26208221300929 |
| Menopausal Sta |                                  | Phone:          | 01070859303    |
| NRS SCORE:     |                                  | Marital Status: |                |
| BMI:           |                                  | Family History: |                |
| CO Morbidity:  |                                  | Reason Of MDT:  |                |

**Clinical Data:**

BX:  R L  
ER:  PR:  HER2:  KI67:

**Sonomammography:****Mets Workup:****History:**

A CASE PF FEMALE PATIENT WITH HTN

ER 8/8 , PR 6/8 , HER2 -VE &amp; KI67 20%

A CASE OF mbc TO LUNG PT RECEIVED AI X 1.5 YRS --> DP BREAST SHIFTED  
TO FASLODEX + CCK4/6 FOR 1.5 YRS TILL 5/2025 THEN STOPPED

NEW IMAGE CT CTCA --&gt; RD OF PULMONARY NODULES 1.6X1 CM

SD OF BREAST MASS

Bone scan --&gt; free

FOR MDT FOR AROMASINE

**Recommendations:****MDT Outcome & Plan**

to continue faslodex ibrance in view of SD even after stopping it

**SIGNATURE**

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|------------------|---------|-----------|-----------|-----------|-------------|
|                  |         |           |           |           |             |



Patient's Name: Safia Abdelmonem Bahy EIDomity  
Date: 2026-05-02  
Referred by: Prof.Dr. CAPwithC  
Hospital: Goustave Roussey International Egypt

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

**Known case of left breast cancer. Compared to the previous study dated 10-1-2026 the current study showed:**

**I-Chest:**

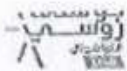
- No appreciable time interval changes regarding the size and number of the previously seen pathological discrete and amalgamated left axillary and subpectoral lymph nodes the largest measures 5.2 x 3.9 cm
- Mild progressive course regarding the size and number of the previously seen bilateral pulmonary nodules largest still seen at the right middle lobe measuring 16 x 10 mm.
- Regressive course since last study regarding the previously seen bilateral ground glass densities.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural or pericardial effusion.
- Cardiomegaly is noted

**II-Abdomen and pelvis:**

- The liver is average in size with homogenous CT texture and regular outline. No definite focal lesions or foci of abnormal enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No sizeable pathologically enlarged abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

**III-The scanned skeleton:**

- Still noted lytic anterolisthesis of L4 over L5 vertebra.
- No worrisome lytic or sclerotic bony lesions.



Patient's Name: Safia Abdelmonem Bahy ElDomity  
Date: 2026-05-02  
Referred by: Prof.Dr. CAPwithC  
Hospital: Goustave Roussey International Egypt

**OPINION:**

- Mild progressive course regarding the previously seen pulmonary nodules.
- Regressive course regarding the previously seen bilateral pulmonary ground glass densities.
- Stationary course regarding the rest of the study.

*Dr.sherif Wagdy*

Name : Safia Abdul-Moneim Bahi Al-Damati

Date : 23-04-2026 08:15

Branch : Dokki

Medical No. : 357840

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : شركة التأمينات

### Tc-99m MDP ISOTOPIC BONE SCAN

**Clinical History:** Female, 63 years old; History of breast carcinoma, complains of bone aches. Evaluation for possible skeletal metastases.

**Technique:** Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time.

### **Findings:**

**The prior bone scan dated 14/12/2024 was revised.**

- o The skeletal system demonstrates rather physiological radiotracer distribution with no focal abnormal areas of increased or decreased tracer uptake to suggest skeletal metastatic deposits on scintigraphic basis.
- o Expected age-related degenerative changes (at different levels of the spine and around big joints) without suspicious metastatic pattern.

### **IMPRESSION:**

- o No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- o Follow-up imaging only if clinically indicated.

Much oblige  
Dr. Alaa Nagi, MD  
Nuclear medicine

|                |                                        |                    |                                  |
|----------------|----------------------------------------|--------------------|----------------------------------|
| Patient Name   | : الأستاذة / صفية عبدالمنعم بهي النمطى | Request No.        | : 136541                         |
| Patient No.    | : 26208221300929                       | Request Date       | : 03/01/2022                     |
| Path No.       | : 41/2022                              | Verification Date  | : 16/01/2022                     |
| Sex            | : Female                               | Financial Category | : أورام دار السلام - نفقة الدولة |
| Age            | : 59 Y                                 | Department         | : Out Patient                    |
| Referring Doc. | :                                      | IHC No.            | :                                |
|                |                                        | Registered by      | : .marwa mohamed yousse          |

## IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast malignant looking mass.

Nature of the specimen: Tru-cut biopsy.

### Gross Examination:

Multiple greyish white tissue cores, the longest measured 1 cm long, totally submitted.

- Procedure: Tru-cut biopsy.
- Localization: Left breast.
- Tumor extent
  - o Number: N/A
  - o Size: N/A
  - o Skin involvement: N/A
- Distance from margins: N/A
- Lymph nodes number and size: N/A
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

### Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GIII (3+3+2)
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Tumor extent
  - o Number: N/A
  - o Size: N/A
  - o Skin involvement: N/A

Verified by :

Presidential Initiative for Women's Health - Pathology Lab  
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name الأستاذ / صفية عبدالمنعم بهي الدماطي  
Patient No. 26208221300929  
Path No. 41/2022  
Sex Female  
Age 59 Y  
Referring Doc.

Request No. 136541  
Request Date 03/01/2022  
Verification Date 16/01/2022  
Financial Category اورام دار السلام - نفقة الدولة  
Department Out Patient

Registered by .marwa mohamed yousse

### IMMUNOHISTOCHEMISTRY REPORT

- Lymph node involvement: N/A
- Resection Margins: N/A
- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

#### Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma GIII

#### IHC:

ER: Strong nuclear reaction in more than 80% of tumour cells. Positive, (Allred score 8/8)

PR: Strong nuclear reaction in about 50% of tumour cells. Positive, (Allred score 6/8)

Her-2: No membrane reaction in tumour cells. Negative, score 0

Ki67: 20%

Prof. Dr. Gamal Abd-El Aty

Prof. Dr. Amal Hareedy

Dr. Menna Atef

Dr. Abdulrahman Ibrahim

Verified by :

Printing Date : 24/01/2022 14:52:30

Page 2 of 2

Printed By : .marwa mohamed yousse

|                                                      |                                                     |
|------------------------------------------------------|-----------------------------------------------------|
| Patient Name : الأستاذة / صفية عبدالمنعم بهي الدماطي | Request No. : 136541                                |
| Patient No. : 26208221300929                         | Request Date : 03/01/2022                           |
| Path No. : 41/2022                                   | Verification Date : 10/01/2022                      |
| Sex : Female                                         | Financial Category : أورام دار السلام - نفقة الدولة |
| Age : 59 Y                                           | Department : Out Patient                            |
| Referreing Doc :                                     | Registered by : .marwa mohamed youss                |

### Pathology Report

Clinical Data: Left breast malignant looking mass.

Nature of the specimen: Tru-cut biopsy.

#### Gross Examination:

Multiple greyish white tissue cores, the longest measured 1 cm long, totally submitted.

- Procedure: Tru-cut biopsy.
- Localization: Left breast.
- Tumor extent
  - o Number: N/A
  - o Size: N/A
  - o Skin involvement: N/A
- Distance from margins: N/A
- Lymph nodes number and size: N/A
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

#### Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GIII (3+3+2)
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Tumor extent
  - o Number: N/A
  - o Size: N/A
  - o Skin involvement: N/A
- Lymph node involvement: N/A

Verified by :

|                 |                                        |                    |                                  |
|-----------------|----------------------------------------|--------------------|----------------------------------|
| Patient Name    | : الأستاذة / صفية عبدالمنعم بهي النمطي | Request No.        | : 136541                         |
| Patient No.     | : 26208221300929                       | Request Date       | : 03/01/2022                     |
| Path No.        | : 41/2022                              | Verification Date  | : 10/01/2022                     |
| Sex             | : Female                               | Financial Category | : أورام دار السلام - نفقة الدولة |
| Age             | : 59 Y                                 | Department         | : Out Patient                    |
| Referring Doc : |                                        | Registered by      | : .marwa mohamed youss           |

### Pathology Report

- Resection Margins:N/A
- Histologic response to therapy: N/A
- Additional pathologic findings:N/A

#### Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma GIII

For (ER, PR, Her-2, Ki67)

Prof. Dr. Amal Hareedy

Dr. Menna Atef

Dr. Abdulrahman Ibrahim

Verified by :



### Women's Health Outreach Program

Name: Safia abd elmiam bahi

Date: 2 / 1 / 2022

#### Full field Bilateral Digital Mammography & ultrasonography

Clinical summary: left retroareolar palpable lump

#### Detailed Findings

##### Technique

- Craniocaudal & mediolateral oblique views
- High resolution ultrasound

##### -left t breast

- ACR b density.
- The left breast shows ill-defined spiculated dense mass lesion located at retro areolar region associated with architectural distortion and mild overlying skin thickening suspecting skin invasion and nipple retraction

##### • US demonstrate :

- The left breast show ill-defined heterogeneous hypoechoic solid mass lesion with spiculated margins and posterior acoustic shadowing and measuring about (3.7 x 3.0 cm) at its maximum diameters with internal vascularity by colour Doppler study. Associated overlying mild skin thickening likely suspected skin invasion
- Multiple enlarged axillary lymph nodes with effaced hila, **largest one measuring about (2.9 x 0.9 cm).**

##### Right breast

- ACR b density.
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs.

##### Conclusion:

- Left breast: (BIRADS 5) left retro areola malignant solid papable mass with ipsilateral pathological axillary lymph nodes as described.
- Right breast: (BIRADS 2).

##### Recommendation

Tru cut Biopsy is done from the left breast mass on today's sitting for histopathological correlation

Dr. Walaa hussein

Best regards,  
Dr. Hedaya nour

NAME: صفية عبد المنعم باهى

DATE: 2024-12-20

**Bilateral Breast Ultrasonography**

- Status of left breast malignancy shows Left breast shows **ill-defined hypoechoic solid mass with ill-defined and speculated border** ( 1.5 x 1.4 cm ) seen at retroareolar region with disturbance of nearby parenchyma ,vascularity inside , no calcification , previous study before treatment , the mass was measures 3,7x3cm
- Retracted nipple, and slight thickened skin
- Multiple left axilla pathological enlarged sizable lymph nodes with loss of fatty hilum , the largest about 3cm
- Right breast shows Normal sonographic appearance. No solid / cystic mass, dilated ducts or abnormal calcifications ,
- Normal appearance of nipple, areola, skin & subcutaneous tissues
- Normal axillary tail
- Free axilla with no evidence of enlarged sizable lymph nodes

**Conclusion**

*Left breast retro areolar malignant solid mass , the mass is reduced in size as regard previous study after treatment with ipsilateral pathological axillary lymph node*

*Normal right breast*

Much Obligated, Yours  
Dr. GHADA IBRAHIM



|                                                   |                   |
|---------------------------------------------------|-------------------|
| Patient Name: Safeya Abd Al Moneim Bahi Aldamatti | Date : 30/09/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis)            | Age : 63 Year(s)  |
| Medical ID: 668/2022                              | Pat. ID: 29908    |

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS  
REVEALED: -****Known case of left breast cancer****Compared to the previous study dated 5-4-2025:****I-Chest:**

- **Stationary course** as regard size of pathological left axillary lymph nodes (level I and II & III)
- Stationary course of the previously seen right lower lung lobe pulmonary nodule reaching 4 mm.
- Newly developed multiple pulmonary nodules, the largest in the left lower lobe measures 17 x 15 mm
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural or pericardial effusion.
- Cardiomegaly is noted

**II-Abdomen and pelvis:**

- The liver is average in size with homogenous CT texture and regular outline. No definite focal lesions or foci of abnormal enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No sizeable pathologically enlarged abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

**III-The scanned skeleton:**

- Tiny sclerotic osseous lesions are seen at right iliac bone and right humeral head, likely bone islands.

**OPINION:**

Status follow up showing the progressive course since the last study regarding the pulmonary metastatic nodules.

Stationary course regarding the rest of the study

**Patient Name:** Safia Abdel Monaem Bahia **Date:** 14/12/2024

**Referring physician:** Prof.

**$^{99m}\text{Tc}$ -MDP BONE SCAN**

**Desired information:** skeletal surveying to detect multiple lesions.

**Technique:** Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi  $^{99m}\text{Tc}$ -MDP:

**Report:**

- No scientigraphic evidence of osseous lesion could be seen all over the surveyed skeleton
- Heterogenous abnormal radiotracer uptake is seen at the spine , more determined at , mid dorsal , upper lumbar vertebrae , likely of Spondylodegenerative changes
  - Marked osteoarthritic changes are seen at , both shoulders , both knees
- Both kidneys are normally seen .

**Conclusion:**

No definite evidence of active osseous lesions could be seen all over the surveyed skeleton .

Thanks for the referral

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR/ Mohamed Emam



Patient's Name:

Safia Abdul-Moneim Bahi Al-Damati

Date:

2026-09-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN  
AND PELVIS REVEALED:**

**I. CHEST:**

- left breast mass lesion showed heterogenous enhancement with associated skin thickening. The lesion measures 54 x 35 mm. multiple left axillary lymph nodes the largest measuring 45 x 33 mm
- Multiple bilateral pulmonary nodules the largest seen in the left upper lobe measuring 19 x 14 mm showed tiny calcific focus
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

**II. ABDOMEN AND PELVIS:**

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

**Opinion:**

- ❖ Known case of left breast cancer showed left breast nodular lesion as well as left axillary lymph nodes.
- ❖ Multiple pulmonary metastatic lesions



Patient's Name:  
Date:  
Referred by:  
Hospital:

Safa Abdul-Moneim Bahi Al-Damati  
2026-09-10  
Prof.Dr. no doctor  
Goustave Roussey International Egypt

Dr.Asser Ahmed

Name : Safa Abdel Moncim Behi  
Branch : Dokki  
Ref.Physician Prof : Goustav Roussey Egypt

Date : 18-09-2026 11:17  
Medical No. : 464359  
تعلق مستشفى : Ins.Company

## NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 64-year-old female with history of left breast carcinoma. Presents with generalized bony aches. Referred for baseline skeletal staging prior to hormonal therapy, compared to prior study dated April 23, 2026.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

### Findings:

- A discrete focal area of increased tracer uptake is noted at the anterior end of the right 7th rib.
- The remaining ribs and sternum show uniform symmetric distribution.
- Homogeneous tracer distribution is noted throughout the skull, thoracolumbar spine, and pelvic girdle without additional focal hyperfixation.
- Symmetric low-grade uptake is noted at both knee joints, consistent with age-related degenerative osteoarthritis.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

### Impression:

- Equivocal focal uptake at the anterior end of the right 7th rib as described, likely traumatic, though a metastatic deposit cannot be entirely excluded on scintigraphy alone; targeted CT is recommended for correlation.
- No scintigraphic evidence of widespread active osteoblastic skeletal deposits.
- Follow up as clinically indicated would be recommended.

Dr. Alaa Nagi

MD Nuclear Medicine

R E C E N T E R C I C S T O S I A G N O S T I C E N T E R



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Safia Abdul-Moneim Bahi Al-Damati  
Branch : Dokki  
Ref. Physician Prof : Goustav Roussey Egypt

Date : 23-04-2026 08:15  
Medical No. : 357840  
تعاقد مستشفى : تعاقد مستشفى

### Tc-99m MDP ISOTOPIC BONE SCAN

**Clinical History:** Female, 63 years old; History of breast carcinoma, complains of bone aches. Evaluation for possible skeletal metastases.

**Technique:** Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time.

#### **Findings:**

**The prior bone scan dated 14/12/2024 was revised.**

- o The skeletal system demonstrates rather physiological radiotracer distribution with no focal abnormal areas of increased or decreased tracer uptake to suggest skeletal metastatic deposits on scintigraphic basis.
- o Expected age-related degenerative changes (at different levels of the spine and around big joints) without suspicious metastatic pattern.

#### **IMPRESSION:**

- o No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- o Follow-up imaging only if clinically indicated.

Much oblige  
Dr. Alaa Nagi, MD  
Nuclear medicine

N I L E D I A G N O S T I C C E N T E R